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Lessons Learned in Residential Care Institution Transition and Closure

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Acknowledgements

We would like to thank the Royal Government of Cambodia and UNICEF for participating in the study as well as all the organizations that took part in the interviews: Cambodia Children's Trust, Children's Futures International, Friends International, Hagar International, Holt, Komar Rikereay, Mith Samlanh, M'lup Russey, This Life Cambodia, and UNICEF for offering their experiences and expertise. We would also like to thank the Better Care Network for their guidance.

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November 2021

DISCLAIMER

This publication was made possible by the generous support of the European Union (EU) and the United States Agency for International Development (USAID). The contents are the responsible of the lead consultant and Save the Children and do not reflect the views of the EU or USAID.

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Abbreviations

CCWC	Commune Committee for Women and Children
DoSVY	Department of Social Affairs, Veterans and Youth Rehabilitation
FCF	Family Care First
KII	Key Informant Interview
MoI	Ministry of Interior
MoSVY	Ministry of Social Affairs, Veterans and Youth Rehabilitation
MoU	Memorandum of Understanding
NGO	Non-Government Organisation
NISA	National Institute for Social Affairs
RCI	Residential Care Institution
REACT	Responsive and Effective Child Welfare System Transformation
SBCC	Social Behavior Change Communication
SOP	Standard Operating Procedure
UNICEF	United Nations Children's Fund
USAID	United States Agency for International Development
WCCC	Women and Children Consultative Committees
3PC	Partnership Program for the Protection of Children

Executive Summary

"An RCI is a centralized hub for children's needs, so it is about decentralizing those support services back into the community" KII3

In 2017 the Royal Government of Cambodia raised concerns about the rapid increase in the number of institutionalized children in Cambodia[1] and as a result set an ambitious target of safely reintegrating 30 percent of children in residential care institutions (RCIs) into families.[2] By 2019, there has been a 43 percent reduction in the number of RCIs in Cambodia and a 59 percent reduction in the number of children in these facilities.[3]

In addition to this progress, COVID-19 caused global ramifications for anyone working in residential care settings. While there are many devastating consequences of the pandemic, some of the benefits have been the opportunity to reflect on the use of RCIs, the consequences of emergency closure, the importance of follow up in reintegration, and the reliance of international volunteers and visitors for the fundraising model of many RCIs. Successful transition of RCIs consists of a number of interrelated key elements that need to be considered at the same time including: the reasons why children are entering residential care, the laws and policies in place, the motivations and funding models of the RCI management and donors, the availability of services within the community including family-based alternative care services, the individual needs of the children in the RCI, and the ability to support reintegration efforts.

Those interviewed highlighted key themes that support successful transition and closure as well as identifying areas that need to be strengthened.

These include:

- *A strong national framework:* The Royal Government of Cambodia has created a strong national framework through their National Plan of Action which has placed pressure on RCIs to transition and has supported successful reintegration. However more work needs to be done to further standardize child protection responses throughout the country. The Child Protection Law which is currently in development will help support this standardization. The Guidelines on Transition and Closure of

[1] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2017). *Mapping of residential care facilities in the capital and 24 provinces of the Kingdom of Cambodia.* MoSVY, Kingdom of Cambodia.
[2] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2017). *Action Plan for Improving Child Care with a target of safely returning 30 percent of children to their families 2016-2018.* (extended to 2020)
[3] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2020). *Report on Results of the Implementation of the Action Plan for Improving Childcare*.

RCIs which are also currently in draft will also further support the interagency agreements between DoSVY, the RCIs, and civil society organizations which will help to outline the shared vision and plan to achieve transition.

- *Clear roles and responsibilities amongst actors:* Further clarity is needed on the roles and responsibilities at the district level to support transition and reintegration. Investment in DoSVY and the CCWCs would help to further empower them to deliver their mandate.

- *Support on the "how"*: RCI directors and management should continue to be supported through learning exchanges and the sharing of tools, policies, and procedures to show how transition can happen safely and contribute to a shared vision.

- *Greater awareness and targeting of messaging:* While many actors in Cambodia have started raising awareness on the importance of family-based care and the harms of institutional care, more needs to be done to target rural communities and donors outside of Cambodia. RCI staff as well as family members and communities have different motivations for their continued support to RCIs. These motivations and fears need to be better understood to target messaging more specifically. More could be learned in applying SBCC techniques to this specific target group.

- *Advocacy and engagement with donors:* Given how influential the donors can be in whether or not transition and closure is successful, the Royal Government of Cambodia and civil society should consider incorporating a donor engagement strategy to form stronger partnerships with advocacy groups in donor countries and join similar initiatives to stop orphanage volunteering such as the Ministry of Tourism's child safe tourism campaign. Lessons could be taken on how donors have moved away from individual child sponsorship programs of specific children towards community-based sponsorship in other contexts.

- *Community-based services:* Effective transition requires addressing the reasons that children are placed in residential care in the first place. Greater investment is needed in community-based services and outreach to at-risk families, particularly following the economic impact that COVID-19 has had on families.

- *Realistic timeframes:* Transition and closure can take time and having the time to plan the transition and develop a shared vision is beneficial to the staff, caregivers and more importantly to the children in their care. Government, donors and civil society actors that are supporting the transition process need to allow for sufficient time and both human and financial resources to support this process.

- *Emergency closure:* Emergency closure can be traumatic to children and very challenging for all actors involved. Even if forced closure is necessary, it should, to the extent possible, be planned and sequenced as long as there is not an immediate risk to children. DoSVY should play a greater role in facilitating emergency closure given the lack of mandate civil society has in this process.

Background

According to a 2017 report by the Royal Government of Cambodia, "the rapid and uncontrolled increase in the number of institutionalized children in Cambodia, traditionally a country with community-based mechanisms for the alternative care of children, has long raised the concerns of the government and child protection workers."[4] Between 2005 and 2010 there was a 75 percent increase in the number of residential care institutions (RCIs) operating in Cambodia.[5]

According to a 2015 mapping by the Royal Government of Cambodia, there were 639 residential care facilities (406 classified as RCIs) with a total of 26,187 children under the age of 18 living in them, and 16,579 children living in the 406 RCIs.[6] The distribution of RCIs across Cambodia is highly uneven with the majority located in areas frequented by tourists and promoted by a phenomenon commonly known as voluntourism. To address these issues in 2016, the Ministry of Social Affairs, Veterans and Youth Rehabilitation (MoSVY) launched an ambitious action plan to reduce the number of children living in RCIs across five key provinces and introduced a moratorium on the registration of new childcare institutions. The Royal Government of Cambodia has created a legally enabling environment with policies, legislation, and targets for the reduction of children in RCIs.[7] This has formed a strong foundation for NGOs and others to build upon and is *closely aligned* with the Government of Cambodia's Action Plan and priorities around care reform.[8] The three-year action plan from 2016-2018 was extended to 2020.[9] According to a review of the action plan in 2019, the number of RCIs had decreased by 43 percent and the number living in them had decreased by 59 percent.

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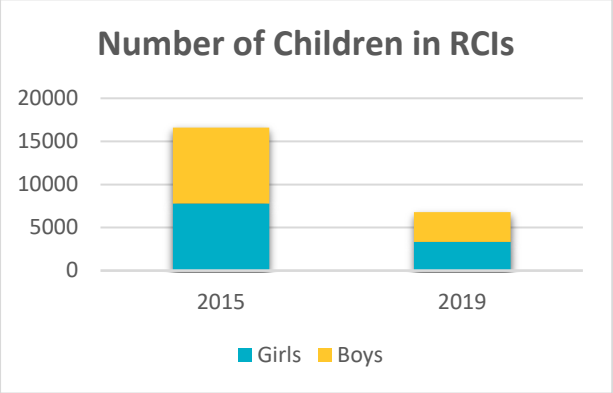
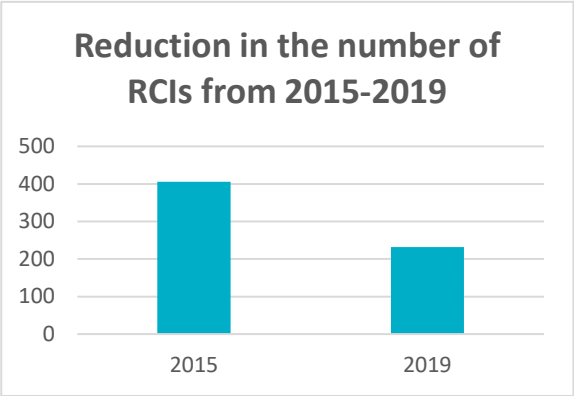
[4] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2017). *Mapping of residential care facilities in the capital and 24 provinces of the Kingdom of Cambodia*. MoSVY, Kingdom of Cambodia.
[5] Better Care Network. (2020). Cambodia Country Care Profile. The Ministry of Social Affairs, Veterans and Youth Rehabilitation and the Better Care Network.
[6] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2017). *Mapping of residential care facilities in the capital and 24 provinces of the Kingdom of Cambodia*. MoSVY, Kingdom of Cambodia.
[7] For a list of relevant laws and policies of reform, please see pages 26-33 Better Care Network. (2020). Cambodia Country Care Profile. The Ministry of Social Affairs, Veterans and Youth Rehabilitation and the Better Care Network.
[8] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2017). *Action Plan for Improving Child Care with a target of safely returning 30 percent of children to their families 2016-2018.* (extended to 2020)
[9] Ibid





Family Care First (FCF), coordinated by Save the Children, was established in 2015 with funding from USAID to bring together organisations using a collaborative approach to decrease the number of children in residential care institutions. In 2019 the European Union provided complimentary funding with the addition of the Responsive and Effective Child Welfare System Transformation (REACT) project to ensure that children's rights and child participation continued to be a vital component of care reform. FCF | REACT is now a network of approximately 60 member organisations, some of whom are funded, working to prevent children from being separated from their families and increase the number of children that are safely and successfully integrated into family-based care. This is done through focusing on four key result areas:

1. National government is supported to roll out child protection legislative and policy frameworks with monitoring systems.
2. Sub-national government authorities in five target provinces are supported to effectively monitor and regulate the quality of protection services and residential care institutions.
3. Girls and boys who have been deprived of parental care and/or separated from their families are provided with appropriate quality alternative care and/or reintegration.
4. Effective measures to prevent family separation are utilized in five target provinces.

Through this effort, according to the Child Tracking Tool, 41 percent of children in the priority provinces who have been reintegrated were either supported by the Government or 3PC and FCF | REACT partners.[10]

[10] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2020). *Report on Results of the Implementation of the Action Plan for Improving Childcare*.

Objective of Study

The objective of this study is to document lessons learned from FCF | REACT members, as well as the literature from others working on transition globally, to create evidence-based recommendations to inform future transitions and closure at scale. This review will be shared broadly with MoSVY, DoSVY, UNICEF, and FCF | REACT members and other interested stakeholders as the Royal Government of Cambodia finalises their Guidelines on Reintegration, Transforming and Closure of RCIs.

Methodology

The identification and documentation of lessons learned for this report was done through two primary methods:

- Desk review of existing documentation/ studies related to RCI transition and closure. This prioritised work done by FCF | REACT members and in Cambodia, but other relevant literature was included as well
- Key informant interviews (KIIs) with key stakeholders involved in, or who have important insights on, RCI transition and closure.

Research Questions

- Under what conditions has transitioning RCIs in Cambodia been the most successful?
    - Under what conditions has closing RCIs in Cambodia been successful (how is this different or similar to transitioning?)
- What are the main steps, stages and process and stakeholders involved in transitioning and/or closing RCIs?
- What approaches are most useful and why?
- What challenges are we facing and how can we best address these?
- What conditions and approaches are different (or the same) for transition versus closure?
- What, if any, additional tools, guidance may be needed?
- How does the experience of transition/closure in Cambodia compare to other countries?

Data Collection and Analysis

Desk Review

Desk review documents were collected in two ways:

1. Cambodian documents identified by the FCF | REACT team based on their knowledge of documentation conducted as well as a request to the larger network for any relevant documents they may be aware of.

2. Global documents, primarily based on a search of the Better Care Network collection of materials and advice from the conveners of the Global Collaborative Platform Transitioning residential care working group, a search for published articles and abstracts, the CPC Learning Network, and the Save the Children Resource Centre. A google search of "transitioning residential care" was also be conducted to ensure nothing was missed.

Documents for the review were included based on their relevance to the topic (i.e. the focus on transition/closure of RCIs) rather than a broader review of general information related to residential care. The desk review was conducted in English and limited to grey literature or practitioner accounts. Particular attention was made to examples gathered from Cambodia and Southeast Asia. In total, 25 documents were reviewed as part of this study.

The findings from this desk review helped inform the KII questions (see Annex 1).

Key Informant Interviews

KIIs were held with key stakeholders involved in – or who would have important insights on – RCI transition and closure. Stakeholders to be included in the KIIs were identified by FCF | REACT based on known involvement in RCI transition or closure. Stakeholders were approached by the FCF | REACT team with the objective of this lessons learned documentation and requested to participate. They were then asked to identify staff member(s) who were best placed to be interviewed and whether they preferred to be interviewed in English or have Khmer translation for the interview.

A formal letter of request for an interview was sent to MoSVY and a representative from the Royal Government of Cambodia was also interviewed as part of this process.

In total, 12 key informant interviews with 19 stakeholders (16 Cambodians/3 expatriates; 47% female/53% male) from 11 organisations were conducted over September-October 2021. Those participating were from the Royal Government of Cambodia, the Better Care Network, Cambodia Children's Trust, Children's Futures International, Friends International, Hagar International, Holt, Komar Rikereay, Mith Samlanh, M'lup Russey, This Life Cambodia and UNICEF.

Analysis

During each interview in-depth verbatim notes were taken, and the interviews were recorded with the consent of those being interviewed. Each interview was conducted separately for each organization with the exception of M'lup Russey and Friends International which were interviewed together with the support of Khmer translation for an hour and a half rather than the one hour allocated to other interviews. The notes were printed out and coded based on key themes that emerged. The researcher then listened to all of interviews again to see if any areas were missing, if any codes needed to be added, and to verify the accuracy of any quotes included. The desk review informed the initial key questions and then each of the documents was reviewed again against the key themes identified through the key informant interviews. These key themes were then used as the headings for the report.

Ethical Considerations

All interviews were confidential unless there was something a specific a stakeholder wanted to have included, however none of those interviewed requested this. The researcher gained verbal consent for each of the interviews as well as consent to record the session. While the interviews were recorded for the purposes of notetaking, they were destroyed when the report was finalised.

Questions were framed around work with RCIs rather than individual children, but if any child safeguarding concerns had arisen, these would have been shared with the safeguarding focal point at Save the Children International for follow up. While this was in place, no safeguarding concerns were shared as part of the interview process. Children were not interviewed as part of this study.

Limitations

This review is specifically focused on the experiences of stakeholders involved in supporting the transition and closure of RCIs and is therefore limited to the perspectives of those interviewed. However, some information from the desk review, including case studies developed by the Better Care Network and findings from focus group discussions held with RCI staff as part of the co-design process for FCF in early 2021, have also captured some perceptions from RCI staff and directors. This study also does not specifically look into implications of COVID-19 on RCI transition and closure, but it is referenced as many of those interviewed touch on these issues, as well as it also being highlighted in the study on the impact of COVID-19 on privately run residential care institutions which was included in the

desk review.[11] While the desk review did highlight the most relevant documents, there have not been many articles written on this subject. With only 25 items, it is highly probable that some lessons learned have just not been published, documented, or shared. As part of this process, a representative from MoSVY was interviewed, however no members of DoSVY or the CCWC were interviewed, and it is very possible that they will have different perspectives based on their experience.

Findings

The Royal Government of Cambodia has made significant progress in supporting the transition and closure of RCIs throughout the country especially through the National Action plan and subsequent enabling legislative and policy environment.

"When the policy came out and they realized that the Government was serious about it… many RCIs were motived to change their approach." KII8

[11] Nhep, R. van Doore, K. (April 2021). *Impact of COVID-19 on Private Residential Care Institutions: Insights and Implications for Advocacy and Awareness Raising*. Better Care Network, Law Futures Centre, Griffith Laws School.

"The Government plan is very important, especially from the Ministry level, the Ministry is promoting this work and now we have the department here to reintegrate children. Sooner or later there will be no more RCIs." KII9

However, successful transition consists of a number of interrelated key elements that need to be consider at the same time including: the reasons why children are entering residential care, the laws and policies in place, the motivations and funding of the RCI management and donors, the individual needs of the children in the RCI, the availability of services within the community including family-based alternative care services, and the ability to support reintegration efforts. As highlighted by Faith to Action:

"*A transition is not the mere closing of residential care. It is a transformation into a new way of conducting business, providing care, and interacting with the community."*[12]

While the majority of FCF | REACT members interviewed are supporting a small number (1-5) of RCIs to transition or close, several have supported more with one having supported 50. Through the literature review, and interviews with key stakeholders, a number of themes emerged on what supports successful transition and closure of RCIs and some of the barriers to successful transition. All of those interviewed discussed at length the importance of working in partnership, specifically with the local and national Government and the role of the donors in supporting or undermining transition. Many highlighted the challenges in changing the mindsets of those involved at various levels, considerations for RCI staff, having realistic timeframes, the challenges and considerations for emergency closure and the importance of support from the community. In the next section each of these areas will be discussed in more depth.

Partnership and Coordination

All those interviewed stressed the importance of working in partnership with the national and local Government (MoSVY/DoSVY), as well as the RCI managers and directors and the civil society organisations that provide various levels of support. Some of those were able to formalize this partnership through interagency agreements between the Government, civil society, and RCI or MOUs with the parties involved. They recommended that these

[12] Faith to Action Initiative. 2016. Transitioning Family Care for Children: A Guidance Manual. Page 16.

agreements clearly outline the roles and responsibilities of each actor and be accompanied by a shared workplan. Some formed a steering group made up of the RCI management, NGO supporting the process and DoSVY while others suggested in retrospect that a steering committee would have been helpful to oversee and drive the process forward. The majority of those interviewed stressed how Government involvement at both the national and district levels have been fundamental in convincing the RCIs to transition.

Many of those interviewed specifically noted that the National Action Plan and commitment to a 30 percent reduction of children in RCIs, helped to motivate RCIs to start the transition process. This was further supported as MoSVY started their inspection process. However, some also highlighted that the ambitious target made it hard to keep up with the demand and some RCI directors struggled to get the support they needed to safely transition.

"To get this task done successfully, we need to work with local governments like the Commune Committees." KII7

"Based on this operational plan, the working group for leading and co-leading, Excellency Chairs for the Women's Commune Council (WCC) based on the action plan… have an RCI meeting and make a plan together, which NGOs want to integrate together, we make a plan together and we provide the technical support to the RCI which want to cooperate with DoSVY.. some cases are managed by the Government, other cases managed by NGOs." KII12

"I think things happened too quickly here. There was a big push, and it was happening. We didn't have all the help immediately, even these guidelines, transition and closure guidelines. We didn't all come together with a strong roadmap at the very beginning that would have helped… some closed and didn't have support." KII8

The literature backs up the need for engaging multiple stakeholders to support successful transition. This includes the Government, but also the children and families, the donors, the staff, and the service providers.[13] Coordination is particularly important given the number of actors within Cambodia responsible for child welfare and child protection with MoSVY having the primary responsibility, but with services overseen and delivered by the District and Commune Administrations which are overseen by the Ministry of Interior (MoI).[14] A study of the impact of COVID-19 on residential care institutions found that 76 percent of RCIs in the

[13] Faith to Action Initiative. (2016). Transitioning Family Care for Children: A Guidance Manual.
[14] Better Care Network. (2020). Cambodia Country Care Profile. The Ministry of Social Affairs, Veterans and Youth Rehabilitation and the Better Care Network.

study experienced a decrease in the number of children in their care with the greatest determinant of reintegration throughout COVID-19 based on pre-existing government gatekeeping mechanisms in place.[15]

Some of the challenges faced initially with reintegration were based on the lack of a standard approach for reintegration and insufficient funds for subnational actors to carry out their roles. MoSVY, with the support of UNICEF and civil society actors, is currently drafting an overarching child protection law supported by Standard Operating Procedures (SOP) for Child Protection. As of November 2021, there were a series of consultative workshops with relevant ministries following a series of sub-national consultations on the child protection law. This law, along with the Guidelines on Reintegration, Transforming and Closure of RCIs will help to further clarify the roles and responsibilities of the various actors in transitioning RCIs, reintegrating children in their care, and investing in community-based services and support. The Guidelines for Reintegration and Reunification of Children from RCIs currently being drafted by MoSVY is intended to support DoSVY and partners to more consistently approach reintegration. It should also support with the establishment of Provincial Reintegration Teams in key provinces to further help to standardize approaches.[16]

In addition to SOPs, MOUs, and interagency agreements, those interviewed also spoke about the practical activities that can support partnership. They mentioned importance of learning exchanges from those that have done it, study visits, and establishing support groups with RCIs that are going through the process to learn from one another.

Coordination and communication were also highlighted as a key component of successful reintegration in terms of case management, family tracing and reunification, but also maintaining good practices of monitoring, documentation, and evaluation.[17]

"We are not doing it alone. We work with the village support groups and we have RCIs, MoSVY and the community people together." KII10

"We believe this is all about working in partnership with the RCI. We're giving them our policies and systems and process and they are adapting them to their needs…. The partnership is critical because you have to have someone partnering with you that is confident in the work." KII15

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[15] Nhep, R. van Doore, K. (April 2021). *Impact of COVID-19 on Private Residential Care Institutions: Insights and Implications for Advocacy and Awareness Raising*. Better Care Network, Law Futures Centre, Griffith Laws School.
[16] Ministry of Social Affairs, Veterans and Youth Rehabilitation. (2018). Guideline for the Establishment of Provincial Reintegration Teams (Kandal, Phnom Penh, Preah Sihanouk, Battambang and Siem Reap).
[17] Wedge, J. 2012. Reaching for Home: The Interagency Group on Reintegration. Global Learning on Family Reintegration in Low and lower-middle income Countries. GHR Foundation.

"We do training, study tour and exchange perspectives from those that have already transitioned, to see the way those that transitioned did it and to share their process and understanding the challenges… It is very helpful. We are not doing it alone, we work with the village support group and we have the RCI, MoSVY, and community people together." KII10

"In the past when we transferred a case, we had to travel, but now between MoSVY and DoSVY, we can transfer the documents through the online system. This can help" KII12

"Some of the children are from far provinces, not only neighbouring provinces so this requires a lot of efforts of the staff." KII9

Changing mindsets and creating shared vision

The concept of changing mindsets and perceptions of RCIs came up in many ways relating to the RCI staff members, the Commune Committees for Women and Children (CCWC), and the children themselves. This is the aspect of transition that interviewees stated takes the most time. Most of the civil society partners started to engage the RCIs through information sessions where they discussed the problems associated with residential care and often showcased other organisations that had transitioned, usually together with MoSVY or DoSVY representatives. Some worked directly with RCIs that they were already engaged with or had worked with in the past. Some organisations shared studies which highlighted the harm of institutional care, while others shared case studies and personal stories of children or care leavers who transitioned out of residential care, while others focused on the business model and made an economic argument for change. This reflects themes from the literature review, especially several case studies,[18] which show the different motivations of RCI directors and the different viewpoints that led them to change their perspective. Some were pragmatic and concerned about the business model while others were motivated by hearing about children whose lives had been transformed through this process, or those that just needed to know what transition would look like and needed to hear examples from those that had successfully transitioned. Still others were motivated by research and academic studies highlighting the harms of institutionalisation on children. In each of those case studies, it was vital that those working with the RCI directors could understand their motivations and concerns to target their messaging more effectively. The Guidance Manual on Transitioning to Family Care for

[18] Better Care Network. 2021. A Case Study of the Process of Change: Transitioning Firefly Orphanage. Better Care Network and Kinnected; Better Care Network. 2021. A Case Study of the Process of Change: Bridges Safehouse. Better Care Network and Kinnected.

Children also supports this with a section on knowing your audience, what motivates them and what their concerns may be.[19]

"Identify RCIs that are really committed. If they are not clear, it is really challenging, and you spend a lot of resources and get no result." KII7

"We can help with reskilling the workforce, but really the desire has to come from the donors or the board itself." KII3

"We have one RCI that wants to join us for transition. We have been working with her for the last two years, just for the reintegration program, but when we did our work together, this will help a lot. She will talk to the donor with the new activities in her mind. She had her plan for after the transition and she brought this plan to talk with the donor." KII9

"Culture eats policy for breakfast and if you asked me what would be the biggest success, it is the move to get RCIs to recognise the importance of involving children in the decision-making process...It is that culture change that drives the transition." KII5

Another aspect of successful transition and closure was on changing the mindsets of community members, including CCWC members, but also the parents and the children. Several of those interviewed spoke about the important role that the CCWCs play in the placement of children and supporting gatekeeping. Several of those interviewed felt that changing the CCWC mindset is a key aspect of effective gatekeeping and preventing children from being placed unnecessarily into the RCIs in the first place.

"They [CCWC] are the gatekeeping mechanism. They are the ones that sign the enrolment to the RCIs. We need to work with these people to stop children being placed into RCIs." KII7

Others talked about the need to further sensitise parents on the harms of institutional care and the benefits of family-based care and how once a child has been in an RCI for a while, it can take time for them adjust to being back with family members or in the community. Still others felt that real change will come in Cambodia when parents and caregivers start to demand for

[19] Faith to Action Initiative. (2016). Transitioning Family Care for Children: A Guidance Manual.

their children to come home and have been changing their approach from focusing on working with RCIs to transition to focus more on building support and services within communities.

"The children are also used to receiving everything from the RCI and they change their lifestyle. They find it difficult to survive in the community. That is why we help care leavers." KII10

"The challenge is the family themselves. They still believe the RCI is the best place for children. Even if they can support the child, they still believe that the RCI is the best place for the child." KII4

"When we think children live in an RCI, it is better for them and when we integrate them into the family, we find a few ways to support them. This is what we learned from the experience we did. If the children live in the family, so we still find a new way, a new method to support them in the family." KII10

According to FHI360 which has created user friendly learning materials on how to develop a social behaviour change communication strategy,[20] one first needs to understand the gaps in knowledge to then design a communication strategy based on that analysis and create a strategy for each specific audience that an organisation is trying to influence. A content brief can help practitioners to identify the audience, what they want the audience (e.g. RCI directors or children and families) to do differently, what is keeping them from doing the desired behaviour, what motivates them and where they get information they trust.

Example Content Brief for Developing a Communication Strategy for Transition and Closure

Audience	What do we want them to do?	Why are they not doing it?	How do we motivate them?	Where do they get information, they trust
RCI Director	Transition their RCI to community-based services	Unclear of how to do it Fear of losing income	Appeal to greater good Showing them how others have successfully transitioned	CCWC, other NGOs, church if a religious organization

[20] FHI360 C-Modules: https://www.fhi360.org/resource/c-modules-learning-package-social-and-behavior-change-communication

			Supporting them to engage their donors	
Family members	Recognize the benefits of family-based care and support their children to be reintegrated	Misperception of benefits of RCIs Lack of services in their community	Awareness raising about the benefits of family-based care, harm in institutionalization Access to support within the community (income generating activities, support for fees for education etc.)	Marketplace Family members

Once that strategy has been designed and the messages have been created, the organisation needs to get feedback from the audience and community to see whether or not the messages are actually getting across and creating the change needed. The importance of reviewing the strategy was seen in the RAPID MERL review in 2018 of an SBCC campaign where the campaign did not appear to affect whether respondents believed that RCIs could result in harm generally, however it did affect whether or not respondents thought RCIs could be detrimental to the emotional development and physical wellbeing of children. Researchers recommended that future campaigns for community members provide more detail on why RCIs can be harmful for children.[21] A study on the attitudes towards residential care in Cambodia conducted by MoSVY and UNICEF reflect these views stating that 71 percent of commune council members and village chiefs thought that residential care was the best solution for children with no parents with 84 percent agreeing that a poor family should send their child to an RCI for education if they could not afford to pay school fees in the village. In this study, very few Village Chiefs had heard of community-based care options, but did feel if these options existed, they would be positive for children. While 61 percent of families agreed that family-based care was better for children, 85 percent also agreed that children in orphanages received a better education compared to those living in a village and that education and the perception of a better future superseded the advantages of home life.[22] While public campaigns have started to shift this mindset and build public support for the closure of RCIs, there is not current data to show that these mindsets have significantly changed yet. A baseline survey conducted in 2019 as the Keeping Families Together campaign relaunched,

[21] USAID. 2018. Rapid Feedback MERL & Cambodian Children's Trust Final Learning Check. Powerpoint Presentation.

[22] Ministry of Social Affairs, Veterans and Youth Rehabilitation. 2011. With the Best Intentions: Attitudes Towards Residential Care in Cambodia.

only 17 percent of community members understood the negative impact of institutionalization on children.

Community support

There was general agreement in the need for more services and support within communities, particularly focusing on poverty reduction and family supports, particularly for families facing multiple challenges like caring for children with disabilities or those with HIV/AIDs. While the provincial, district and commune level government often require support of civil society organisations, more work has been done to increase commune council budgets for social services, as highlighted in the Social Service Expenditure Guidelines in 2018.[23] However, one interviewee (KII12) noted that gaps still remain with the social assistance policy still limited for families with children under the age of two and while there is some reintegration support available for children being reintegrated from Government RCIs, this funding is not available for all reintegrating children.

"There has been too much time focused on a crisis response when we are missing out on a lot of prevention and early intervention. It is much cheaper with better outcomes. That might be the natural evolution." KII3

"If we want community-based care services, we need the potential services for children and their families, and this service needs to be sustainable. If the service is not sustainable, just one year or two year[s], our funds cut off so that the children that still need support, what should they do? We need to make clear what services to provide those children… and we need them [family] to have a strong income" KII12

"You need to address why those children were placed in the first instance and unless you address the root causes, you won't make any progress." KII3

"We increased our community foster care so if other clients can't go home, they go into foster care." KII6

"We need a bottom-up driver where the families are supported to care for the children themselves. Now they feel RCIs are better able to provide the support. They aren't doing this

[23] Better Care Network. (2020). Cambodia Country Care Profile. The Ministry of Social Affairs, Veterans and Youth Rehabilitation and the Better Care Network.

out of lack of love or ability to care for them. This is much more successful than driving it from the top down." KII3

Role of the Donors

While most of the work civil society in Cambodia has been doing is focused on targeting the RCI managers and directors, many organisations interviewed still find it challenging to engage the donors. This was the main barrier that many organisations found in helping RCIs to transition and the reason most cited as to why an RCI transition process failed to progress. Overseas donors are the main funders of residential care in Cambodia. According to the Study on Attitudes towards Residential Care almost 100 percent of funding for RCIs came from foreign funding. While some Cambodians had started RCIs, they relied on funding from overseas support.[24] Many RCIs use tourism to attract funders. Some collect funding from individual donors or churches with some relying on just one or two individuals for funding.

"We provide the reintegration, but each RCI is donor driven. They have to have the confidence to talk to the donors. The management has to be very confident and convincing, and most managers are cautious. 'If I change my direction, I am not confident that you will still be supporting my programmes. They are very dependent on the tourist and visitors and how these donors can understand and support." KII9

"I think it is a problem if we want all RCIs to 'change their plan', I use the word 'change their plan', they don't feel good because their donor says if they have RCIs, they provide the funds, if they don't provide RCIs, they don't provide the funds" KII12

"It is harder to raise funds with children in centres versus children in the community. It is a harder sell for most people and really the donors are blissfully unaware that the children aren't orphans." KII3

"They [RCI management] changed their commitment and their plan because their funders and their board of directors decided not to transform." KII7

"The biggest thing during the pandemic for the world, RCIs become more and more dependent. They used to rely on individual visitors coming to their centre, but now it is solely

[24] Ministry of Social Affairs, Veterans and Youth Rehabilitation. 2011. With the Best Intentions: Attitudes Towards Residential Care in Cambodia.

dependent on donors. We need to build the capacity of the management to speak with donors… not only in Cambodia, but in the world." KII9

There were many examples of work that had started with RCI directors and then stopped due to the lack of donor interest or support. In addition, the types of donors were also referenced with some explaining how much harder they found it to work with informally run RCIs and those that rely on sponsorship through multiple donors. Those that transitioned, including the organisations that transitioned themselves, all received backing from their donors or were able to find different donors to support. In several of the successful examples, the RCI directors were confident at fundraising themselves and also had a clear vision of what their new model of community care would look like. Fundamentally, transitioning requires a new business model. Faith to Action states that along with a plan and a vision, it is important to review financial and human resources and the resources needed to support and sustain the transition.[25] Donors are clear partners in the transition process which can be very challenging for civil society partners and local government given how protective and untransparent RCI directors can be about their donors and funding model.

Additionally, there were examples of RCIs that had to close immediately due to lack of funding or lack of donor support which created a crisis situation for the children in their care. Residential care often operates without long-term financial stability and some donors continue to be concerned that funding directed at families will not be spent on children.[26] Even those that had transitioned, recognised that fundraising for family and community-based care can be much harder, especially if donors were used to a sponsorship model allowing them to have an ongoing relationship with a specific child.

A study on the impact of COVID-19 on RCIs found that the control of fundraising and the ability to access alternative sources of funding to transition were the greatest determinants of whether a participant was willing and able to consider changes to their model of care including transition and closure. Eighty one percent of participants questioned in a study on the impact of COVID-19 on residential care noted that funding had been negatively impacted, including the ability to hold fundraising events, public speaking or through hosting volunteers or visitors in their facilities. The study cited anecdotal reports from Cambodia that COVID-19 restrictions

[25] Faith to Action Initiative. (2016). Transitioning Family Care for Children: A Guidance Manual.
[26] Ministry of Social Affairs, Veterans and Youth Rehabilitation. 2011. With the Best Intentions: Attitudes Towards Residential Care in Cambodia.

had increased the openness of orphanage directors to explore family-based care and long-term reintegration.[27]

Given how influential the donors can be in whether or not transition and closure is successful, the Royal Government of Cambodia and civil society should consider incorporating a donor engagement strategy to form stronger partnerships with advocacy groups in donor countries and join similar initiatives to stop orphanage volunteering such as the Ministry of Tourism's child safe tourism campaign.

Considerations for RCI Staff

The biggest concerns that the key informants shared from the RCI staff and directors was in their funding model and the role of their staff. Some used funding to retrain the staff or reallocate them to community-based activities. Many of the NGOs helped to train the RCI staff in case management, reintegration techniques, foster care and how to manage data and fill out the relevant Government forms on each child. Some NGOs were involved in completely reskilling the staff to other professions of their choice. The majority of those interviewed felt that investing in the RCI staff made the transition process much easier and supported the transition to having skilled case workers and carers in the community.

This is also supported by the work of MoSVY to strengthen the social service workforce through a competency-based system. By the end of 2021 MoSVY and the National Institute of Social Affairs (NISA) will endorse Social Service Workforce training modules and training plans for 2022In focus group discussions with RCI staff as part of the FCF co-design process the need for support to help with family tracing and reunification and reintegration from NGOs was highlighted.[28]

"We have been able to understand the impacts of residential care on children and what the benefits would be to living with families. They [organisations supporting transition] have to be able to understand the facts, but then they have to be able to explain this to the RCIs. They also need to explain how their job would be secured." KII9

"Some RCIs are concerned about their jobs. The staff may lose their job, so we need to create other project work with the director" KII5

[27] Nhep, R. van Doore, K. (April 2021). *Impact of COVID-19 on Private Residential Care Institutions: Insights and Implications for Advocacy and Awareness Raising*. Better Care Network, Law Futures Centre, Griffith Laws School.
[28] Focus group discussions with RCIs – Co-Design 2021 FDGs finding presentation.

"I know it is hard for our staff. We had to cut down our number of staff and they faced many challenges in their personal income… We help them find a job and do references and recommendations for them. Most of our caregivers have been placed in other organizations running shelters. Some have helped while running the foster care programme." KII6

"Some of the staff resigned, some were promoted to social worker and some, we asked them to be the foster parent of children." KII4

"Some of the staff were trained as social workers and still work with us today" KII3

Realistic timeframes

Almost all of those interviewed stressed the importance of taking the time to engage and build relationships with RCIs. Many of those that had transitioned themselves said that it had taken between 3-5 years, with the majority of time needed to get everyone on board and, create a vision of what their new model would look like, and then in working to reunify children with their families.

Most started with gatekeeping and creating individual care plans for each child, some with a traffic light system identifying those that could easily be reunified first, and then shifting in providing services based on their needs, whether that was temporary foster care, longer term foster care placement or semi-independent living. Some of the organisations stopped taking in new children but then waited for the children that could not be reintegrated in their centres to age out of care before closing their centre completely. While some organisations were concerned RCIs did not provide appropriate follow up for children that had been reunified, others, particularly those working with RCIs engaged in sponsorship felt the RCIs wanted to support the children far into adulthood.

"Most RCIs haven't done reunification plans properly. They want to take care of the children for the rest of their lives, like graduating from university and marriage." KII10

In retrospect, when asked what they would have done differently if they could do the process again, either in their own transition or in supporting others to transition, most interviewees said that they would take more time to develop a plan and shared vision before moving into reintegration. Given that children come from all provinces to the RCIs, some found challenges in ensuring that appropriate services and support were offered in their province of origin which highlights the need for a coordinated approach.

"It's a long journey, and that's what it takes." KII5

"It took several years for us as well, but once we came to a decision, once we had a decision it was 3-4 months to transition from the board approval and the communication to all staff and support office as well as donors." KII6

"Two years is not enough to work with RCIs on transition. Reintegration takes time (3-6 months). If we do it fast, we cannot raise our voice very high and DoSVY does not have much funding for transition… It is not just a commitment for 1-2 years. It should be at least 3 years." KII9

"When we partner with them, it takes 2-3 years. We provide capacity building for the director, but then if they change their position or a new staff member comes in, we have to support them." KII10

"It isn't a quick fix, and we don't turn that funding tap off completely as we would be left with a crisis to deal with." KII3

"I think things happened too quickly here. There was a big push, and it was happening. We didn't have all the help immediately. Some [RCIs] closed and didn't have the support." KII8

"Anyone who says they can do it quickly; they aren't doing it properly. This takes a bit of time. It is changing cultures." KII5

Emergency Closure

Those involved in emergency closure were very concerned about the pace of closure and the impact on the outcomes for children. They felt there was more work needed with RCIs to plan in advance, particularly for those RCIs that had failed inspection, but may not pose an immediate threat to children. Some of those that had worked on reintegrating children from RCIs following emergency closure felt that the Government more generally, and DoSVY specifically, needed to take more responsibility in making this transition happen smoothly. This is echoed in the Draft Guidelines for the Transition and Closure of RCIs[29] which highlight that emergency forced closures can be very traumatic for children as there is no time to prepare or consult with the child prior to emergency placement. It is also very challenging for both DoSVY and NGOs to find appropriate placements for the children. While forced emergency

[29]Ministry of Social Affairs, Veterans and Youth Rehabilitation. Currently unpublished. Guidance for the Transition and Closure of Residential Care Institutions.

closures may sometimes be warranted if there is an immediate risk to the children in their care, more often non-emergency forced closures are more appropriate which allow for planning and ensure a better transition for the children in their care.

"Some run out of resources and need to close immediately and don't have reserve budget for reintegration." KII2

"It is very different from an RCI transition. RCI transition is a lot better because there is more time to prepare the strategy and better for the child." KII7

"For emergency closure, [there is] a committee and technical staff that go through the process, who is responsible for the children, specific programs to transition to, someone that can help. We work with local authority at the subnational level." KII10

Recommendations

According to a 2017 report by the Royal Government of Cambodia, "the rapid and uncontrolled increase in the number of institutionalised children in Cambodia, traditionally a country with community-based mechanisms for the alternative care of children, has long raised the concerns of the government and child protection workers."[30] As a result the Cambodian Government set an ambitious target of safely reintegrating 30 percent of children in residential care institutions (RCIs) into families.[31] By 2019, there has been a 43 percent reduction in the number of RCIs in Cambodia and a 59 percent reduction in the number of children in these facilities.[32]

In addition to this progress, COVID-19 caused global ramifications for anyone working in residential care settings. The study on the impact of COVID-19 stated that 90 percent of their respondents felt COVID-19 had catalysed reflection and created an opportunity to consider and implement changes. COVID-19 caused global ramifications for anyone working in residential care settings. While there are many devastating consequences of the pandemic, one of the benefits has been the opportunity to reflect on the use of RCIs, the consequences of emergency closure or reintegration and the reliance volunteers and visitors for fundraising. Those interviewed all had different views and experiences, there were common themes that emerged that can help to form future practice.[33]

It is clear that legislation and mandates from the national government are incredibly important in framing the need for transition and driving the ability to open, regulate and close residential care institutions. While the MoSVY has been quite clear through their national action plan about their ambition, greater clarity is needed at the district and provincial levels with clear roles and responsibilities for the CCWCs and local government to both oversee and support reintegration, particularly when there is no civil society partner able to play this role. These roles can be clarified further in the Guidelines on Transition and Closure of RCIs when endorsed by MoSVY. SOPs or interagency agreements between DoSVY, the civil society partner, and the RCI in helping to create a clear plan of action, set responsibilities and a shared vision to support in the transition process. In addition, DoSVY and the CCWCs need to have

[30] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2017). *Mapping of residential care facilities in the capital and 24 provinces of the Kingdom of Cambodia.* MoSVY, Kingdom of Cambodia.
[31] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2017). *Action Plan for Improving Child Care with a target of safely returning 30 percent of children to their families 2016-2018.* (extended to 2020)
[32] Ministry of Social Affairs, Veterans, and Youth Rehabilitation (2020). *Report on Results of the Implementation of the Action Plan for Improving Childcare.*
[33] Nhep, R. van Doore, K. (April 2021). *Impact of COVID-19 on Private Residential Care Institutions: Insights and Implications for Advocacy and Awareness Raising*. Better Care Network, Law Futures Centre, Griffith Laws School

appropriate funding to carry out the roles they have been allocated to support in transition and reintegration.

A number of those interviewed raised the challenge of trying to work with and influence unregistered RCIs. The Royal Government of Cambodia has created clear regulations and penalties for unlawfully running RCIs, however as the regulations have now been in place for some time, they now need to be more rigorously enforced to support compliance.

In addition to mandates, RCIs need support in learning from the many civil society organisations within Cambodia that have already successfully transitioned. Learning exchanges and sharing tools, policies and procedures can complement the legislative framework and help reduce the fear and uncertainty RCI directors and staff may feel as they start the transition process.

Transition and closure can take time and that having the time to plan the transition and develop a shared vision is beneficial to the staff, caregivers and more importantly to the children in their care. Government, donors, and civil society actors that are supporting the transition process need to allow for sufficient time and both human and financial resourced (at least 3 years of funding) to support this process. This includes funding for CCWCs and other sub-national actors to carry out their role effectively

Greater consideration and planning should be considered to support emergency closure to reduce the negative impact on children. While forced emergency closures may sometimes be warranted if there is an immediate risk to the children in their care, more often non-emergency forced closures are more appropriate which allow for planning and ensure a better transition for the children in their care. Those interviewed felt that DoSVY needs to play a greater role in supporting this process.

While advocacy and campaigning has helped to raise awareness within Cambodia, more data is needed to determine how much community member's perspectives have changed and which mechanisms are most helpful in facilitating this. It is clear from both the interviews and literature review that different messages are needed depending on the motivations and understanding of the RCI staff as well as community members. More work needs to be done to target the CCWCs and rural communities that may have had less exposure to the messaging. This can help to prevent placement in the first place, and support successful reunification.

Given how influential the donors can be in whether or not transition and closure is successful, the Royal Government of Cambodia and civil society should consider incorporating a donor engagement strategy to form stronger partnerships with advocacy groups in donor countries

and join similar initiatives to stop orphanage volunteering such as the Ministry of Tourism's child safe tourism campaign. Many donors have already successfully transitioned from using sponsorship of specific children as a fundraising model towards sponsoring children in a community. Lessons could be learned from how donors were encouraged to move from one form of giving to supporting communities.

Successful reintegration ultimately addresses the reasons that children were placed into RCIs in the first place. Greater investment in community-based services and outreach to at-risk families will provide much needed support to help keep children in families and reduce unnecessary separation.

ANNEXES

Annex I: Key Informant Interview Questions

1. How are you currently supporting RCIs to transition and/or closure?

2. How are the RCIs selected?
   - Did someone at the RCI approach you/your organisation, were they selected by DoSVY, or did you approach them?
   - At what stage was the RCI at when you approached them/they were assigned to you? (i.e. raising awareness/learning; planning and preparation phase, implementation phase)

3. How many RCIs have you been able to support?
   - Of these, how many have you supported through the conclusion of providing residential care services? Did they transition or close?
   - How many are still in process and what stage are they at?

4. Whom within the RCIs do you work with and how?
   - Looking back, are there other actors that you wish you had engaged but didn't?

5. What supports do you think RCI's need for transitioning and closure and what is currently provided?

6. Do you know where the RCIs that you are working with get their funding from?
   - If yes, where?

7. Do you know who founded the RCIs you are working for and why they were founded?

8. Do you think all RCIs can or should fully transition to providing other services? Why or why not?

9. How did your organisation approach the different steps in the transition process?

10. How long has the transition process with different RCIs taken?

11. In your experience, what factors helped RCIs to move forward in the transition process?
    - And what made it more difficult to move forward?

12. Have you ever had a situation where you became concerned about the welfare of children in the centre or concerned about the motives of the stakeholders in the RCI?
    - If yes, what did you do? If no, what do you think you would do?

13. How is the process similar or different for emergency closure?

14. Reflecting back on the process:
    - What advice would you give to others supporting RCIs to transition?
    - Think of an example that has been successful, what do you think lead to that success?
    - What has been the most challenging part of supporting RCIs to transition?
    - Is there additional advice, tools or guidance that you think would help you or other organizations assist in the transition process?
    - Is there anything that you have changed about the way you are working? Why?

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